



Healthcare Worker Immunisation and Health Policy

1. Scope

Site	Operational Area	Applicable to
Armada Kalamunda Group	All services	All disciplines

2. Purpose

The purpose of this document is to outline the immunisation and health assessment and screening requirements of all healthcare workers/employees (HCW's) and volunteers, within the Armada Kalamunda Group (AKG). This document is in accordance with the [East Metropolitan Health Service \(EMHS\) Immunisation for Health Care Workers](#), [EMHS Pre-Employment Health Screening Policy](#), and [Health Department Western Australia \(HDWA\) Health Care Worker Immunisation Policy](#).

3. Policy

- All employees and volunteers will be risk assessed, screened, and vaccinated (if required) against vaccine preventable diseases specified in this policy prior to commencement of duties.
- HCW's immunisation and screening requirements are based on risk assessment, categorised according to the level of patient contact and risk of exposure to blood and body fluids ([Section 5 - Risk categorisation for screening and immunisation](#)).
- Certain HCW's are required to undertake screening for Methicillin-resistant *Staphylococcus aureus* (MRSA) and prior exposure to Tuberculosis (TB).
- All employees and volunteers have a responsibility to comply with this policy and complete immunisation and screening as required.
- Managers/supervisors must ensure all immunisation and health screening of employees have been met prior to commencement of duties.
- Unremunerated clinical arrangements are not included within the scope of this policy but are covered by the [EMHS Unremunerated Clinical Agreements policy](#).

4. Pre-employment/Placement Requirements

4.1 New Employees and Volunteers

"NO SCREEN, NO START"

(If clearance is not obtained from Staff Health/Infection Prevention and Management and OSH Departments, **employment cannot proceed**)

- All prospective employees must complete the [N10 – Pre-Employment Health Assessment form](#) and provide all required supporting evidence (*proof of immunity, vaccination*)

statements, serology test results, immunisation, and health screening summary from previous employer) based on their risk category.

- All prospective volunteers must complete the Pre-Commencement Health Screening Form and provide all required supporting evidence (*proof of immunity, vaccination statements, serology test results, immunisation, and health screening summary from previous employer*).

4.2 Students and Trainees (Medical, Nursing, Midwifery and Allied Health)

- Universities, academic institutions and HCW education providers are responsible for the screening and vaccination of students, as part of the Student Clinical Placement Agreement, and ensure that students have been immunised in accordance with the requirements specified in the [Health Care Worker Immunisation Policy](#).

4.3 Agency Staff

- Relevant employment agencies are responsible for the screening and vaccination of their staff, and only provide contract staff who have a documented screening and vaccination history consistent with the requirements specified in the [Health Care Worker Immunisation Policy](#).

4.4 Unremunerated Appointees (Visiting Medical Practitioners, Clinical Observers, Emeritus Consultants, Clinical Assistants, Mentoring Arrangements, Supervisors)

- Refer to "[EMHS Unremunerated Clinical Agreements Policy](#)"

5. Risk Categorisation for Screening and Immunisation

Category	Risk	Examples	Recommended Vaccines
A	Direct contact with blood or body substances This category includes all persons, who have physical contact with, or potential exposure to, blood or body substances.	Dentists, medical practitioners, nurses, allied health practitioners, health care students, laboratory staff, maintenance engineers who service equipment, sterilising service staff, cleaners, and staff responsible for the decontamination and disposal of contaminated materials.	▪ Hepatitis B x 3 doses followed by serological confirmation of immunity ▪ Influenza yearly ▪ Pertussis (dTpa) within last 10 years (and completed primary course DTPa for tetanus component) ▪ Measles, mumps, rubella (MMR) x 2 doses ▪ Varicella x 2 doses
B	Indirect contact with blood and body substances Rarely have direct contact with blood or body substances. These employees may be exposed to infections spread by the airborne or droplet routes but are unlikely	Catering staff and ward clerks	▪ Influenza yearly ▪ Pertussis (dTpa) within 10 years ▪ Measles, mumps rubella (MMR) x 2 doses, ▪ Varicella x 2 doses

Category	Risk	Examples	Recommended Vaccines
	to be at occupational risk from blood borne diseases.		
C	Minimal patient contact Occupational groups that have no greater exposure to infectious diseases than do the general public. The exact nature of job responsibilities should be considered when deciding immunisation requirements and all staff should be encouraged to be fully vaccinated.	Administrative/Office clerical staff, gardening staff and kitchen staff.	Influenza yearly
Laboratory staff	May have additional vaccination requirements if they are working with, or may be exposed to, specific viral or bacterial organisms, e.g. anthrax, Q fever, anthrax, meningococcal C, poliomyelitis, Japanese encephalitis, typhoid, yellow fever	Laboratory staff	Laboratory staff should also receive the vaccines noted in category A
Health care workers who work with remote Indigenous communities in WA, NT, and SA should receive vaccines that are recommended for all HCW plus hepatitis A vaccine			

6. Immunisation and Screening Requirements

Immunisation	Details and minimum acceptable evidence of immunity
Hepatitis A	- HCWs whose duty places them at increased risk of exposure to hepatitis A are required to provide documented evidence of completed age-appropriate course of hepatitis A vaccine or serological evidence of hepatitis antibodies. - Non-immune HCWs will be offered hepatitis A vaccination.
Hepatitis B	- HCWs directly involved in patient care and /or at risk of blood and body fluid exposure should be vaccinated. The risk of acquiring hepatitis B through occupational exposure is assessed at pre-employment. - Documented evidence of a completed, age-appropriate course of hepatitis B vaccine, including documented evidence of post-vaccination hepatitis B surface antibody (ant-HBs) (> or + to 10mIU/ml on serology or presence of anti-HBs). - For those who have completed a course of hepatitis B vaccine but have no documented evidence of conversion or demonstrated levels $\leq$ 10mIU/ml of anti-HBs should be followed up in accordance with the recommendations in the Australian Immunisation Handbook.

Immunisation	Details and minimum acceptable evidence of immunity
Measles, Mumps & Rubella (MMR)	- HCWs including volunteers will have either serological testing to determine immunity to MMR or documented evidence of 2 MMR vaccinations at least 1 month apart. - Those born before 1966 are considered immune to measles. - If not immune, HCWs will be offered the vaccine. Those non-immune must not attend to patients with suspected or proven measles
Varicella (Chicken pox)	- HCWs including volunteers will have either serological testing to determine immunity to Varicella (VZV) or documented evidence of 2 VZV vaccinations at least 1 month apart. - If not immune HCWs will be offered the vaccine. Non-immune staff must not attend to patients with suspected or active chicken pox or shingles.
Pertussis	- HCWs including volunteers with patient contact should provide documentation of receiving a pertussis containing vaccine. - One documented dose of adult diphtheria, tetanus, and pertussis (dTpa) vaccine in the last 10 years.
Influenza	It is recommended that all HCWs and volunteers receive the influenza vaccination on a yearly basis to maintain ongoing protection against the influenza virus.
COVID-19	Recommendations for COVID-19 vaccinations refer to Communicable Disease Network Australia (CDNA) National guidelines for public health units .

Screening	Details
MRSA for further details click link below: MRSA Screening and Management of Healthcare Workers	<u>New HCWs</u> - All HCWs, who will have clinical contact, are required to have their risk status for MRSA assessed as part of a pre-commencement process. - HCWs who have direct patient contact and have worked in a healthcare facility or residential-care facility outside Western Australia (WA) or have been hospitalised outside of WA, in the previous 12 months, require MRSA screening prior to commencement of duties. - MRSA screening swabs can be collected outside of WA, if the HCW has not worked since collection of the screening swabs. <u>Current HCWs</u> - HCWs who perform clinical duties in any HCF outside of WA, including volunteer placements, and are returning to work, are to have MRSA screening performed on their return. - HCWs can continue clinical duties pending results. <u>Visiting HCWs</u> - Any visiting HCWs who wear surgical attire and are assisting or observing in an operative / procedural setting and are visiting for less than five days are exempt from screening requirements.

Screening	Details
	- Any visiting HCWs who will be involved in clinical contact for a period greater than five days, require screening prior to placement. <u>Additional HCW Screening</u> - Screening of HCWs should be considered when a single strain outbreak continues, despite adherence to IPC measures i.e., standard, and transmission-based precautions. - Any HCW who develops an exfoliative skin condition or skin, and soft tissue infection (SSTI) is to seek immediate medical advice and have MRSA screening from any skin lesions or wounds. It is recommended that HCWs with these skin conditions do not perform clinical duties until the condition has resolved, however each case should be assessed by an individual risk assessment undertaken by the Advisory team. - There is currently no evidence to support the routine screening of HCWs who have been employed in WA residential care facilities prior to employment in the acute care setting, or for those HCWs who work across both care settings. - All HCWs should be educated on the increased prevalence of MRSA in residential care, the subsequent increased risk of becoming colonised with MRSA and the importance of hand hygiene in minimising this risk. <u>Screening Specimen Samples</u> - Nasal Swab - Throat Swab - Any skin lesions or wound <u>How to screen</u> - Rotating a single swab, 2-3 times around the inside of the nostril, using the same swab for both nostrils - swabbing the posterior pharynx and lateral walls of the pharynx i.e., 'tonsillar' area, without touching the buccal mucosa or tongue - swabbing any discharging wounds, ulcers, or skin lesions. Swabs collected from dry sites e.g., nostrils or non-discharging lesions, should be pre-moistened with sterile normal saline or sterile water. Swabs collected from moist sites e.g., discharging wounds, do not need to be premoistened. <u>Management of HCW colonised with MRSA</u> Refer to MRSA Screening and Management of Healthcare Workers.
Tuberculosis for further details click link below: Guidelines for Tuberculosis Control in Western Australia	- HCWs will be assessed for risk of prior tuberculosis infection and predicted risk of future occupational exposure during the pre-employment screening process. - Indications for Latent Tuberculosis Infection Testing are outlined in the Guidelines for Tuberculosis Control in Western Australia. - New HCWs deemed at risk require an Interferon Gamma Release Assay (IGRA) blood test in the absence of Mantoux results, or previous IGRA blood test results with evidence of appropriate medical follow up where required.

Screening	Details
	HCWs returning a positive IGRA blood test will be referred to the Anita Clayton Centre (ACC) for follow up. Referral to ACC in the absence of signs and symptoms suspicious of TB will not delay employment.

7. HCW Immunisation Program

- HCWs are responsible for their own immune status to Vaccine Preventable Diseases (VPDs) and should be up to date with their immunisations. All HCWs are offered vaccinations for the VPDs at AKG.
- Vaccines are only administered by registered HCWs who have completed an accredited immunisation training program.
- Vaccines given at AKG Staff Health Clinic are recorded on the Australian Immunisation Register and local staff database.

7.1 Staff Immunisation Clinic

Location	Operating Hours	Contact Details
Armadale Hospital – Staff Health Clinic -Goline House (Room 39)	Monday to Friday (by appointment)	<u>Phone:</u> 9391-2691 or 0437934053 <u>Email:</u> AHSStaffhealth@health.wa.gov.au
Kalamunda Hospital – Infection Prevention and Management Office	By appointment	<u>Phone:</u> 9257 8100 or 9257 8198 <u>Email:</u> kdch.infectioncontrol@health.wa.gov.au

7.2 Vaccine consent

HCWs are provided with information about the contra-indications, side-effects and benefits of vaccination and are required to sign a consent form prior to receiving the vaccination.

7.3 Refusal of Vaccine

- HCWs are to be advised of the potential consequences if they refuse reasonable requests for immunisation. Such advice and refusal should be documented.
- Vaccine refusal can be managed by ensuring appropriate work placements, work adjustments and workplace restrictions.
- A vaccination refusal form obtained from the Staff Health team must be completed.

7.4 Non-Responders/ Medically Contra-indicated to vaccine

AKG has a duty of care to HCWs who remain non-immune through failure to seroconvert and/or have medical contra-indication to the vaccine (medical certification provided to Staff Health). Vaccine contra-indication or non-responders may be managed by ensuring appropriate work placements, work adjustments and work restrictions or referred to immunology were required.

7.5 Adverse vaccination events

- HCWs are required to report any adverse events following immunisation (AEFI) to their vaccine provider.
- Where an AEFI has occurred, it is required by legislation, they report to a medical practitioner within 14 days of the event. Refer to the [WA Vaccine Safety Surveillance \(WAVSS\)](#).

7.6 Existing HCWs vaccination catch-up program

- All existing HCWs are being offered screening for immunity to VPD's.
- Staff Health Team will monitor the existing staff's compliance and request for immunisation and/or screening were appropriate.
- Staff Health Team provide quarterly reports on the staff immunisation compliance and the non-compliant staff list for follow up to the ward/department managers.

7.7 Staff Health Summary/Immunisation Record

- HCWs who require evidence of their vaccinations and immunity status can obtain a Staff Health Summary (SHS).
- Request for SHS can be made by sending email to Staff Health Clinic/Infection Prevention and Management ([See Section 7.1 Staff Immunisation Clinic for contact details](#)) with the following details:
 - Name
 - Date of Birth

8. Health Care Worker performing exposure prone procedures

Exposure prone procedures (EPPs) are procedures where there is a risk of injury to the HCW resulting in exposure of the patient's open tissues to the blood of the HCW. These procedures include those where the HCW's hands (whether gloved or not) may be in contact with sharp instruments, needle tips or sharp tissues (spicules of bone or teeth) inside a patient's open body cavity, wound or confined anatomical space where the **hands or fingertips may not be always completely visible**.

- HCWs who perform EPPs must take reasonable steps to know their BBV status and should be **tested for BBVs at least once every three (3) years** as per [CDNA Guidelines](#).
- Registered HCWs who perform EPPs must declare whether they are complying with the [CDNA Guidelines](#) when they are renewing their annual health practitioner registration. HCW are not required to provide the results of testing to the health practitioner Board that has registered them or to the Australian Health Practitioner Regulation Agency (AHPRA).
- HCWs will be assessed at Pre-Employment and offered confidential testing as required. HCWs also have the option of arranging testing with a practitioner of their choice.

All BBV testing performed for HCWs at AKG is kept strictly confidential and accessible only by authorised personnel.

9. HCWs Living with Blood-Borne Viruses (BBV)

- HCW living with a BBV have a duty of care to declare their status before commencement of duties as part of the Pre-employment Health Assessment (E. Blood Borne Virus Screening – under Part B of N10 form).
- HCWs will be referred to the Infectious Diseases (ID) Consultant or General Practitioner prior to commencement of duties.
- If the HCWs duties require modification it will be under the direction of the ID Consultant.

10. HCW newly diagnosed with BBV

- When diagnosed with a BBV, HCWs must cease performing all EPPs immediately and seek appropriate medical care.
- HCWs with a BBV may return to performing EPPs once they meet the criteria set out in the CDNA Guidelines.

11. HCWs with skin conditions (dermatitis)

HCWs with skin conditions (cracked skin, dry skin, hand dermatitis, reactions to hand sanitizers, gloves, and face masks) should seek advice from the Staff Health Team, as this can be an infection risk to both the staff and the patient under their care.

12. HCW with Respiratory Virus Infection (Influenza-like illness/COVID-19)

- HCWs with symptoms of Influenza-like illness/Acute respiratory infection, should undertake COVID-19 testing and self-exclude from work until symptoms resolve.
- For information on management of HCWs with COVID-19, refer to [Coronavirus Disease2019 \(COVID-19\) Infection Prevention and Control in Western Australian Healthcare Facilities](#).

13. Pregnant Healthcare Worker

Pregnant HCWs must strictly adhere to standard and transmission-based precautions during clinical patient contact. Avoidance of contact with certain known infections will reduce the risk of acquiring infection. It is important that pregnant HCWs are aware of pathogens, the risk they impose and the preventive measures that can be implemented

14. Occupational Exposure to Blood and Body Fluids

For information on management of occupational exposure to blood and body fluids, refer to [Occupational Exposure to Blood and Body Fluids \(AKG-PRO-0315-13\)](#)

15. Legislative context

The following documents are required to give effect to this policy:

- *Western Australia Health Act 2016*
- *Western Australia Occupational Safety and Health Act 1984*
- *Western Australia's Work Health and Safety Act 2020*
- Health Department of Western Australia (HDWA) - [Management of Occupational Exposures to Blood and Body Fluids in Healthcare Settings \(Guideline 0008 / 4 May 2022\)](#)
- East Metropolitan Health Service (EMHS) - [Immunisation for Health Care Workers \(EMHS: 11\)](#)

- EMHS [Pre-Employment Health Screening \(EMHS: 71\)](#)
- (HDWA) - [Health Care Worker Immunisation Policy \(OD 0049/07\)](#)
- (HDWA) – [Staff Member Influenza Vaccination Program Policy](#)

16. Supporting information

The following documents support the implementation of this policy:

- CDNA – [Australian National Guidelines for the Management of Healthcare Workers Living with Blood Borne Viruses and Healthcare Workers who perform Exposure Prone Procedures at risk of Exposure to Blood Borne Viruses](#)
- Department of Health and Aged Care – [Australian Immunisation Handbook](#)
- Hand Hygiene Australia – [Hand Care Issues](#)
- Health Department of Western Australia (HDWA) – [Guidelines for the Screening and Management of MRSA in Healthcare Workers.](#)
- National Health & Medical Research Council (NHMRC) [Australian Guidelines for the Prevention and Control of infection in healthcare \(2019\)](#)
- North Metropolitan Health Service (NMHS) - [Guidelines for Tuberculosis Control in Western Australia](#) (September 2019)
- [Occupational Exposure to Blood and Body Fluids \(AKG-PRO-0315-13\)](#)
- Occupational Dermatology Research and Education Centre – [Healthy Skin for Healthcare Workers](#)
- [Tuberculosis Management Guideline \(AKG-GUI-0512-19\)](#)

17. Compliance, monitoring and evaluation

Compliance against this policy will be monitored by the Preventing and Controlling Infections Committee (PCIC) including:

- Quarterly review of staff immunisation status as per audit schedule

18. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 3 – Action 3.01 Integrating clinical governance
- Standard 3 – Action 3.15 Workforce screening and immunisation

19. Policy owner

Enquiries relating to this policy may be directed to:

Department: Staff Health/Infection Prevention and Management

Email: AHSStaffhealth@health.wa.gov.au

20. Review

This policy will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
Version 5	21/12/2023	21/12/2026	Scheduled review – minor amendment

21. Authorisation

This policy has been authorised and issued by the Director Nursing and Midwifery.

Authorised by	Helen Fullarton – Director Nursing and Midwifery
Authorisation date	20/12/2023
Published date	21/12/2023

Appendix 1: Recommended work restrictions for HCWs with specific infections

Disease / Infection	Work restriction	Duration
Conjunctivitis	No contact with patient or patient care environment	Until discharge from eyes ceases
COVID-19	Exclude from duty	Refer to Coronavirus Disease 2019 (COVID-19) Infection Prevention and Control in Western Australian Healthcare Facilities
Cytomegalovirus infection	No restriction Pregnant staff have the option of not working with these patients	N/A
Diarrhoeal disease (including various bacteria, parasites and viruses such as <i>Salmonella</i> , <i>Shigella</i> , <i>Campylobacter</i> , <i>Giardia</i> and <i>Rotavirus</i> etc.)	No contact with patient, patient care environment or food handling	Until 24 hours after diarrhoea ceases or 48 hours for confirmed norovirus
Hepatitis A Virus (HAV)	No contact with patient, patient care environment or food handling	Until 1 week after onset of jaundice or 2 weeks after onset of symptoms (if not jaundiced)
Hepatitis B Virus (HBV)	Contact Staff Health Clinic for advice	
Hepatitis C Virus (HCV)	Contact Staff Health Clinic for advice	
Herpes Simplex Virus (HSV) Genital	No restriction	
Human Immunodeficiency Virus (HIV)	Contact Staff Health Clinic for advice	
Human Parvovirus B19	No restriction Avoid contact with pregnant staff for 5 days after rash onset	
Infected lesions (Bacterial)	Cover with waterproof dressing. Seek advice from Staff Health Clinic. May need to avoid direct patient care	Until lesion is healed
Influenza-like Illness (ILI)	Exclude from duty	Until symptoms resolve
Measles (Suspected or Confirmed) Post exposure, non-immune	Exclude from duty Exclude from duty or re-allocate to low-risk area depending on individual assessment by Staff	For 7 days after onset of rash From 5 th day after exposure and until 21 days after last exposure and/pr 7 days after rash appears.

Disease / Infection	Work restriction	Duration
	Health Clinic and on duty Clinical Microbiologist	
MRSA	No contact with patients	Until decolonisation treatment has commenced, provided they have no lesion.
Meningococcal disease	Exclude from Duty	Until 24 hours after commencing chemo prophylaxis
Post exposure to infected oral secretions	No restrictions	
Mumps (Suspected or confirmed) Post exposure, non-immune	Exclude from duty	For 9 days after onset of salivary gland swelling (parotitis) From 12 th day after exposure until 26 th day after last exposure, and/or 9 days after onset of parotitis
Pediculosis (Head lice)	Exclude from duty	Until the day after topical treatment
Pertussis (Active)	Exclude from duty	Until 5 days after antibiotic treatment or 21 days after onset of coughing
Post exposure, non-immune	No restriction – prophylaxis recommended	
Ring worm	Exclude from duty	Until 24 hours after commencement of anti-fungal treatment
Scabies	Exclude from duty	Until the day after topical/oral treatment
Shingles (Herpes Zoster) Localised	No direct patient care contact to pregnant women, neonates, immunocompromised patients, burns patients and patients with extensive eczema	Lesion must be covered with waterproof dressing. If lesions cannot be covered (Ophthalmic zoster), must not provide direct patient care.
Disseminated	Exclude from duty	Until all lesions are dry and crusted
Post exposure, non-immune	Individual case assessment required	
Streptococcal infections	Lesions must be covered with a waterproof dressing. If lesions cannot be covered, no direct patient care or food handling. If pharyngitis or tonsillitis, avoid direct patient contact	Until 24 hours after commencement of appropriate antibiotic treatment
Tuberculosis (Pulmonary/Laryngeal) Active	Exclude from duty	Until cleared by Staff Health Clinic in consultation with Anita Clayton Centre

Disease / Infection	Work restriction	Duration
Latent Tuberculosis (No symptoms of active TB)	No restrictions	
Varicella Zoster Virus (VZV) Chicken pox – Suspected or Confirmed) Post exposure, non-immune	Exclude from duty Exclude from duty or re-allocate following assessment by Staff Health Team	Until all lesions are dry and crusted From 10 th day after exposure until 21 st day after last exposure (after 28 th day if VZV is given)